

Designed to guide initial investigations including CT head when presenting complaint is headache <2 weeks

DO NOT use if

- GCS <15
- T>37.9° C
- WCC >12
- New cognitive dysfunction
- New neurological deficits
- Haemodynamic instability
- Pregnancy at 20 weeks gestation or over, with known preeclampsia or ED BP >139/89

Disclaimer:
This is a clinical template; clinicians should always use judgment when managing individual patients

Re-approved by ED guidelines committee on 25Jun25
Review next due: Jun 2030 . Trust Ref: C70/2015

Patient details

Full name

DoB

Unit number

(use sticker if available)

① Are any SAH high-risk indicator flags present?

☐ **Yes** - at least one of the below

- TLOC (transient loss of consciousness) ☐
- Neck stiffness (defined as inability to touch chin to chest or, if supine, raise head 8cm off trolley or bed) ☐
- Diplopia ☐
- Seizure ☐
- Previous SAH ☐
- Family history of SAH ☐
- Known unruptured cerebral aneurysm ☐
- History of intracranial aneurysm in at least one 1st-degree relative ☐
- Autosomal dominant polycystic kidney disease ☐
- Ehlers-Danlos syndrome type IV ☐
- Fibromuscular dysplasia ☐
- Pseudoxanthoma elasticum ☐

☐ **No** - none of the above

② Are other criteria for SAH testing present?

☐ **Yes** - as at least one of the below

- Aged 40 years or more ☐
- Therapeutic anticoagulation ☐
- Thunderclap headache (i.e. peaking within 5min) ☐
- C/O neck pain or stiffness ☐
- Headache onset during exertion ☐
- Known brain tumour ☐

☐ **No** - as none of the above

③ Pain-onset-to-CT interval

Date pain started

Time pain started

Time CT done

Interval between headache onset and CT request
(record in hours if <48, otherwise record in days)

④ Are any other indications for brain imaging present?

☐ **Yes** - one or more of the below

- Suspected intracranial bleed**
 - Therapeutic anticoagulation ☐
 - Platelets <100 ☐
 - Known coagulopathy ☐
 - Re-attending ED after recent head injury (unless CT done at previous attendance and normal) ☐
- Suspected intracranial malignancy**
 - History of brain tumour ☐
 - History of malignancy known to metastasise to the brain; i.e.
 - Breast ☐
 - Lung ☐
 - Kidney ☐
 - Thyroid ☐
 - Melanoma ☐
 - Hodgkin's lymphoma ☐
 - History of malignancy **AND** aged <20 ☐
 - Change in personality ☐
 - Vomiting without obvious cause ☐
- Suspected raised intracranial pressure**
 - Ventriculoperitoneal shunt ☐

☐ **No** - none of the above

Features (usually affecting one eye only) include

- Sudden onset of pain
 - Blurred vision or 'halos' around lights
 - Red eye
 - Mid-dilated non-reactive pupil
 - Corneal edema (hazy cornea)
- Headache, nausea, and vomiting may be associated features

Obtain vital signs and bloods (including plasma viscosity and CRP if aged >50), and control symptoms

Is acute glaucoma suspected?

Refer for urgent review by ophthalmology

Did patient have transient loss of consciousness (TLOC) at the start of headache?

Did headache peak within 1h?

>2 identical episodes over >6 months, OR return after CT+LP already done for same headache episode?

SAH rule-out needed

SAH investigations needed (see boxes 1 & 2)?

SAH excluded

Other indications for CT (see box 4)?

No imaging required

CT head

Any new CT abnormalities?

- If spontaneous intracerebral haemorrhage, refer to stroke team as per [ED stroke proforma](#)
- If [unexpected!] SAH, manage as per UHL guideline 'Subarachnoid haemorrhage in adults suspected: Investigation and initial management', Appendix B
- For any other findings, discuss with your ED senior

Proceed to 'Headache II' ED proforma

CT head **ASAP**
ideally within **6h** from the onset of headache and always within **1h** of arrival

Provide effective pain relief, including opioids if needed
Work out interval between pain onset and CT in box 3

Proceed to box 5 on next page once CT report is available

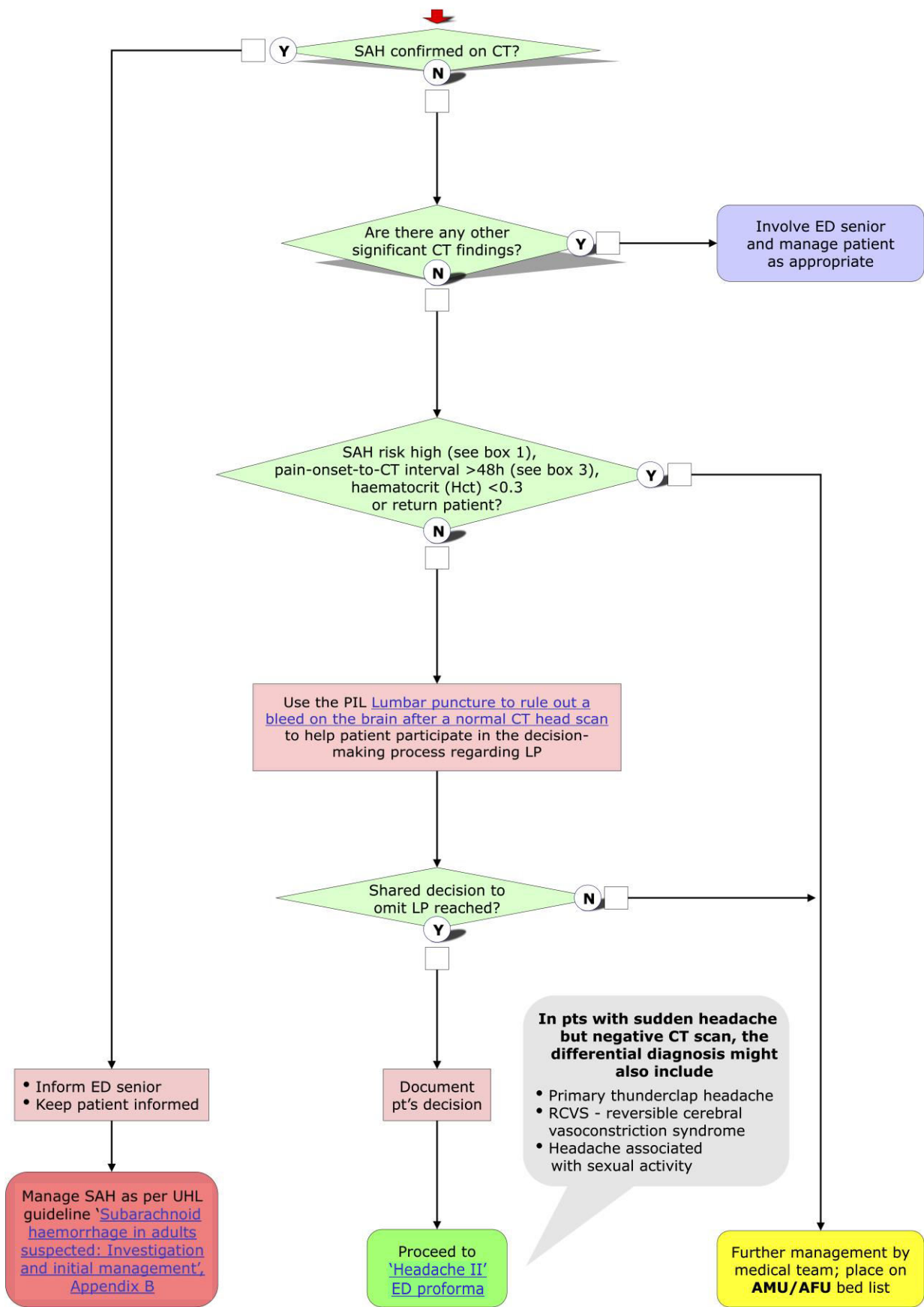
Patient seen by

Print name

Signature

Role

⑤ ED management of patients with suspected SAH following CT report



Patient managed by

Print name

Signature

Role